Patient Education and Counseling 148 (2026) 109581



Contents lists available at ScienceDirect

Patient Education and Counseling

journal homepage: www.journals.elsevier.com/patient-education-and-counseling





Women's screening preferences shift after being informed about the 2024 USPSTF breast cancer screening guideline

Tamar Parmet [a,b,1,*], Grant Yoder [a,h], Kirsten McCaffery [f,2], Jolyn Hersch [f,3], Marilyn M. Schapira [d,e,4], Carmen L. Lewis [a,c,5], Joseph N. Cappella [d,6], Channing Tate [a,c,7], Brad Morse [a,c,8], Laura D. Scherer [a,g,9]

[a] Adult and Child Center for Outcomes Research and Delivery Science, University of Colorado School of Medicine, Aurora, CO, USA
[b] Department of Psychology, University of Colorado Denver, Denver, CO, USA
[c] Division of General Internal Medicine, University of Colorado Anschutz Medical Campus, Aurora, CO, USA
[d] Department of Medicine University of Pennsylvania, Philadelphia, PA, USA
[e] Philadelphia VA Medical Center, Philadelphia, PA, USA
[f] Sydney School of Public Health, Faculty of Medicine and Health, University of Sydney, Sydney, NSW, Australia
[g] Division of Cardiology, University of Colorado Anschutz Medical Campus, Aurora, CO, USA
[h] Department of Health and Behavioral Sciences, University of Colorado Denver, Denver, CO, USA

ARTICLE INFO

Keywords:
Breast cancer screening
Health communication
Patient education
Shared decision-making
Decision aids

ABSTRACT

Introduction: In 2024 the United States Preventive Services Task Force (USPSTF) revised its breast cancer screening (BCS) guidelines to recommend that all women screen biennially from age 40–74, replacing its prior recommendation that women ages 40–49 engage in informed decision-making about when to begin BCS. We examined how communicating information about the new guideline influenced women's screening preferences and views on what information women should receive before screening.

Methods: This study included 301 women who were previously enrolled in a longitudinal study where they were exposed to a decision aid informing them about BCS harms and benefits and the prior USPSTF recommendation. Participants completed an online survey including an informational intervention that presented information about the 2024 USPSTF recommendation. Outcomes included screening preferences pre- and post-intervention and ratings (1 =not at all important to 4 =very important) of information considered important to know before screening.

Results: Information about the new guideline influenced participants' screening preferences. Post-intervention, compared with pre-intervention, participants preferred to screen at their current age (86% vs 81%; $p < 0.001$) and biennially thereafter (45% vs 33%; $p < 0.001$). Participants believed it was important for women to be informed about the benefits of screening (M=3.77), their personal risk of breast cancer (M=3.75), and about overdiagnosis (M=3.48) and false positives (M=3.61).

Conclusions: Being informed about the 2024 USPSTF mammography guidelines shifted screening preferences to be consistent with the new guideline. Even though the new guideline deemphasizes informed decision-making, women still wanted to be informed about BCS harms and benefits, as well as their personal cancer risk.

* Correspondence to: 1890 North Revere Court, Aurora, CO 80045, USA.
E-mail address: Tamar.parmet@cuanschutz.edu (T. Parmet).
[1] 0000–0002-2425–0181
[2] 0000–0003-2696–5006
[3] 0000–0001-5225–6639
[4] 0000–0002-3775–9462
[5] 0000–0002-5473–2229
[6] 0000–0002-1998–4826
[7] 0000–0001-6635–9794
[8] 0000–0002-1080–3961
[9] 0000–0002-8660–7115

https://doi.org/10.1016/j.pec.2026.109581
Received 10 December 2025; Received in revised form 2 March 2026; Accepted 9 March 2026
Available online 10 March 2026
0738-3991/© 2026 Elsevier B.V. All rights are reserved, including those for text and data mining, AI training, and similar technologies.

Practice implications: Clinicians and health systems may consider pairing clear and directive information about the new BCS guidelines with brief information about BCS benefits and harms and personal risk, to support guideline-concordant and informed screening decisions.

1. Introduction

In 2024, the United States Preventive Services Task Force (USPSTF) revised its breast cancer screening (BCS) guideline to recommend that all women begin screening starting at age 40 and continue biennially until age 74. [1] This change contrasted with their 2009 [2] and 2016 [3] guideline, which recommended that women between the ages of 40–49 engage in individual decision-making with their physicians to determine what age to begin screening prior to age 50. [4] Although the USPSTF has consistently emphasized the importance of informed decision-making at all levels of their recommendations,[5,6] and stated that patients have a right to be informed about their medical choices, [6] the language used in 2024 guidelines may be perceived by patients and clinicians as deemphasizing individual decision-making.

The 2024 guideline change was driven by new modeling studies showing a 2% increase in the incidence of breast cancer each year and projections that starting biennial screening at age 40 would save about 20% more lives from breast cancer.[1,7] This change was also made to address racial disparities in breast cancer, particularly that Black women are 40% more likely to be diagnosed with and to die from more aggressive forms of breast cancer. [1,8,9] The shift away from individualized decision-making raises important questions about how women interpret, internalize, and act upon cancer screening guideline recommendations, especially those who may be aware of the benefits and harms of BCS. This is particularly salient given research showing that communication about breast screening recommendations can influence patients understanding of guidelines, their screening preferences, and their adherence to screening programs. [10–12]

In 2021, our team launched a yearlong longitudinal study [13] to examine how a decision aid (DA) [14,15] impacted women's breast cancer screening preferences and screening discussions with their doctor. The 2024 guidelines were released after recruitment was complete and while participants were in the follow-up portion of the study. This timing provided an opportune scenario to evaluate the impact of this guideline change among women who were uniquely well-informed about BCS and the previous BCS guideline. Utilizing this unique cohort of women, we conducted a study to evaluate how the new BCS guideline impacted their screening preferences and beliefs about what women should be told prior to screening. We aimed to answer three research questions. First, how do women's screening preferences change after being informed that the USPSTF now recommends screening for all women starting at the age of 40 (RQ1)? Second, what factors influence women's screening preferences (RQ2)? Third, do women believe it is important to be informed about both BCS benefits and harms, despite the new guideline's reduced emphasis on informed decision-making (RQ3)?

2. Methods

2.1. Participants

We conducted a survey with a cohort of women who were originally recruited into our ongoing longitudinal study exploring the impact of a DA, that educated women about the benefits and harms, on their screening preferences and future screening discussions with their physicians. Participants were recruited from the University of Pennsylvania and University of Colorado Health Systems. Women were eligible to participate in the parent study if they were between the ages of 39–49, had an upcoming well visit appointment with a primary care or family medicine provider, no known BRCA 1 or 2 genetic mutations, and no prior history of breast cancer.

A total of 466 women were enrolled into the longitudinal study, in which an initial survey was administered 1–3 weeks before a clinic visit and contained the DA. A second survey was administered within 2-weeks post-clinic visit, and asked questions about what was discussed during that visit related to BCS. Two additional surveys were administered 3 months post clinic visit, and 1 year post clinic visit. All participants were informed via email of the guideline change on May 8th, 2024 (8 days after the USPSTF announced the new finalized guidelines). At this point, recruitment was complete, and all participants had finished the first survey, which contained the DA. All enrolled participants were then invited to participate in a new follow-up survey about the guideline change. Participants received a $25 gift card as compensation for completing this survey.

2.2. Survey design and intervention

Participants completed a 20-minute electronic survey delivered via email. The survey included several outcomes that are not presented in this article, as the primary goal of this report is to describe how the new guideline changes impacted participants' screening preferences and beliefs about what women should know prior to engaged in BCS.

Participants first reported on their mammogram history and current screening preferences regarding screening age and frequency (details below). Next, participants were provided with information about the USPSTF guideline change using text from the USPSTF FAQ website, [16] with minor edits to decrease the text literacy level. Specifically, participants received the following information. 1. What the USPSTF is now recommending, 2. How this is different from the prior recommendation, 3. Why the USPSTF changed its recommendation, 4. Why the USPSTF does not recommend screening annually, 5. What the USPSTF recommends for women with dense breasts, and 6. What research questions still need to be answered.

Half of participants were randomized to a condition in which they were also told that the overall balance of screening benefits and harms "did not change much compared to the data that informed the previous USPSTF guideline." However, that additional information had no effect on any subsequent survey measure, and so it will not be considered further within this report. A complete copy of the survey and information presented is found in Supplementary Material **A**, and null results for the effect of randomized conditions is available in Supplementary Material **B**.

After reading the presented information, participants reported on their screening preferences again, the topics that influence those screening preferences, and what they think is important for women to know before screening.

2.2.1. Screening preferences

Participants reported their preferred age to start (or continue) having regular mammograms and how often they would like to receive mammograms. The first question has been used in prior research [14] and asked women to select an option that most closely reflected their screening preferences: I am going to start (or continue) having regular mammograms this year, I am going to wait until I'm older but before age 50 to have my first (or next) mammogram, I am going to wait until I am 50 to have my first (or next) mammogram, I do not plan to have a mammogram in the future at any age, or other with free text box to provide clarification. Second, participants were asked if they would prefer to have mammograms every year or every 2 years, assuming their insurance would pay for screening. Participants could indicate a

preference for every year, every 2 years, not sure, an intent to not have a mammogram, or "other" with a text box to explain.

2.2.2. Topics influencing screening preferences

Participants were asked to rate the importance of a list of factors that might have influenced their reported screening preferences about when and how often to screen. Specifically, participants rated the importance of the old USPSTF recommendation, the new USPSTF recommendation, their personal breast cancer risk, their family history of cancer, a need to find out information about their health, what their doctor recommends, their fear of cancer, what their family members think, the harms of screening, the benefits of screening, their ability to pay for their mammogram, their ability to get to and from their mammogram, and the pain of getting a mammogram in influencing their screening preferences. Ratings were measured on a 4-point scale (1 =not at all important to 4 =very important).

2.2.3. What women think is important to know before receiving screening

Participants were asked a series of questions to determine what they believe is most important for women to be educated about prior to getting a mammogram. For example, participants were asked to rate (1 =not at all important to 4 =very important) the importance of being informed about what a false positive is and separately rate the importance of being informed about how often false positives occur (this was repeated for being informed about screening benefit and overdiagnosis). Participants first rated how important it is for *all women to be told* this information, and then were asked to rate how important it is for them *personally* to know this information.

2.3. Analytic plan

Screening age preferences were reduced to three categories: screen at current age; wait to screen (until an older age but prior to 50, screen at age 50, or never screen); and "some other screening plan" which allowed a textbox response. Two members of the study team reviewed the "some other screening plan" responses and recategorized several responses that clearly belonged in the other listed categories (e.g., "planning on getting a mammo within the next 6 months…" was recoded to the "screen at current age"). This process was repeated for "other" responses to the question about screening frequency. Two chi-squared analyses examined change in screening preferences from pre- to post-intervention to explore whether participants' preferred age to screen and frequency of screening changed in response to the guideline change.

Frequency distributions were used to determine how important certain topics were to women in deciding at what age and how often to screen. Frequency distributions were also used to illustrate what participants believe is important for women to be told about certain screening-related topics (e.g., how often false positives happen, what overdiagnosis is, personal risk). This analysis was repeated to determine how important participants believe it is for them, personally, to know about each of those topics.

3. Results

Data were collected from July to August 2024. A total of 301 participants completed the survey and were included in analyses. Table 1 shows the demographic characteristics of the sample.

3.1. Research question 1: how do women's screening preferences change after being informed that the USPSTF now recommends screening for all women starting at the age of 40?

Fig. 1 shows that more participants wanted to screen at their current age after reading about the new guideline (N = 258 post-intervention vs 247 pre-intervention) rather than delaying screening (N = 42 post-intervention vs 30 pre-intervention) compared to before they received this information $X^2(4) = 242.13$, $p < 0.001$. Fig. 2 shows that more participants wanted to screen every 2 years (N = 136 post-intervention vs N = 102 pre-intervention) rather than every year (N = 139 post-intervention vs N = 173 pre-intervention) after getting information about the new guideline compared to before they received this information, X^2 (16) = 569.92, $p < 0.001$.

Table 1
Sample characteristics.

Total	N = 301
Age *Mean (SD)*	43.3 (2.9)
Race (more than 1 race possible)	
White or European American	206 (68.4%)
Black or African American	73 (24.3%)
Asian or Asian American	15 (5.0%)
American Indian or Alaskan Native	6 (2.0%)
Native Hawaiian or Other Pacific Islander	1 (0.3%)
Other Race	15 (5.0%)
Not recorded	0
Hispanic Ethnicity	
Yes, Hispanic	27 (9.0%)
No	274 (91.0%)
Education	
Less than high school	4 (1.3%)
High school diploma or GED	18 (6.0%)
Trade school	7 (3.3%)
Some college, no degree	30 (10.0%)
Associate's degree (AA, AS, etc.)	12 (4.0%)
Bachelor's degree (BS, BA, etc.)	88 (29.2%)
Master's degree (MA, MPH, etc.)	84 (27.9%)
Doctoral/Prof. degree (PhD, MD)	58 (19.3%)
Income	
Less than $20,000	16 (5.3%)
$20,000-$29,999	12 (4.0%)
$30,000-$39,000	9 (3.0%)
$40,000-$49,999	15 (5.0%)
$50,000-$59,999	11 (3.7%)
$60,000-$74,999	18 (6.0%)
$75,000-$99,999	34 (11.3%)
$100,000-$149,999	50 (16.6%)
$150,000 +	118 (39.2%)
Prefer not to say/ Unknown	18 (6.0%)
Insurance	
No	2 (0.7%)
Yes	293 (97.3%)
Unknown	6 (2.0%)

Note. GED=General Educational Development

3.2. Research question 2: what topics influence women's screening preferences?

Fig. 3 shows that the topics rated most important in influencing participants' screening preferences were the benefits of screening (M=3.64, SD=0.630), personal risk of breast cancer (M=3.61, SD=0.710), and their family history of cancer (M=3.48, SD=0.811). The pain of getting a mammogram (M=1.61, SD=0.995), the old USPSTF recommendation (M=2.13, SD=0.957), and what their family members think they should do (M=2.17, SD=0.995) were least important.

3.3. Research question 3: do women believe it is important to be informed about both BCS benefits and harms, despite the new guideline's reduced emphasis on informed decision-making?

All the presented topics were, on average, evaluated as being either important or very important. Frequency distributions demonstrated that "what the benefit of breast cancer screening is" (M=3.77, SD=0.494), "how often breast cancer screening benefits happen" (M=3.74, SD=0.504), and "information about personal risk of getting breast cancer" (M=3.75, SD=0.475) were the most important topics – according to participants' opinions – for *all* women to be educated about prior to getting screened. The pattern was similar for women's ratings of

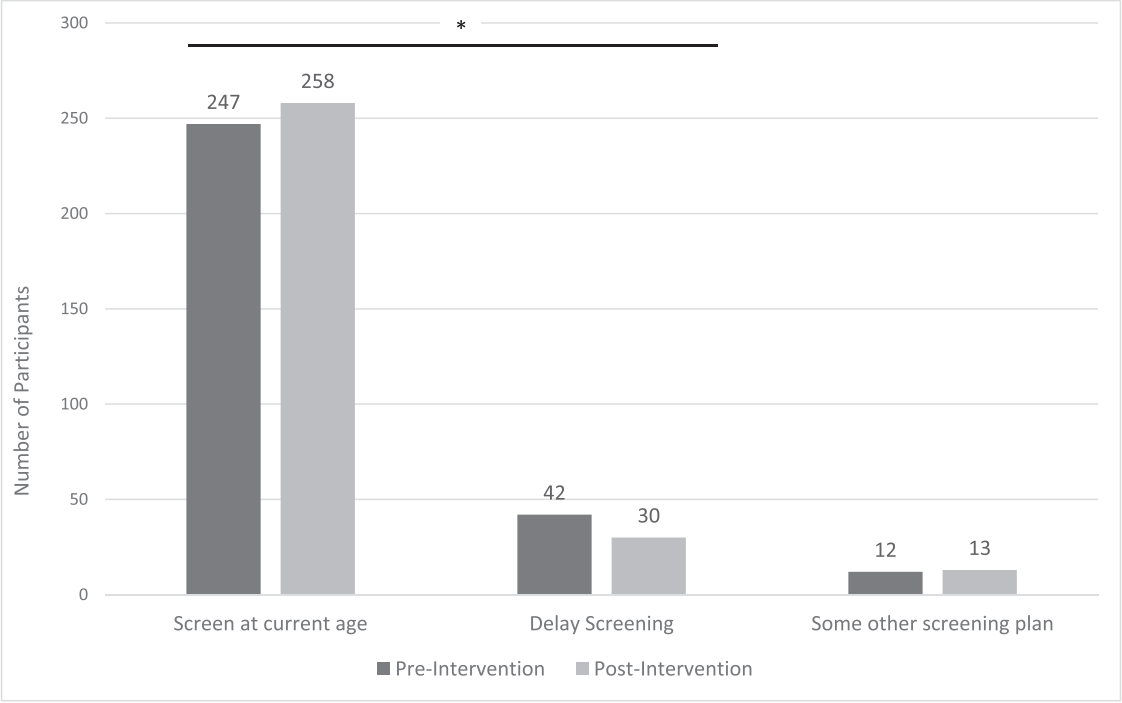


Fig. 1. Screening Age Preference Pre vs. Post Intervention. Note. The horizontal bar indicates that significant differences were observed between participants pre-intervention and post-intervention preferences to screen at their current age or to delay screening. After reading information about the new guideline change, participants were significantly more likely to want to screen at their current age. *p < 0.001.

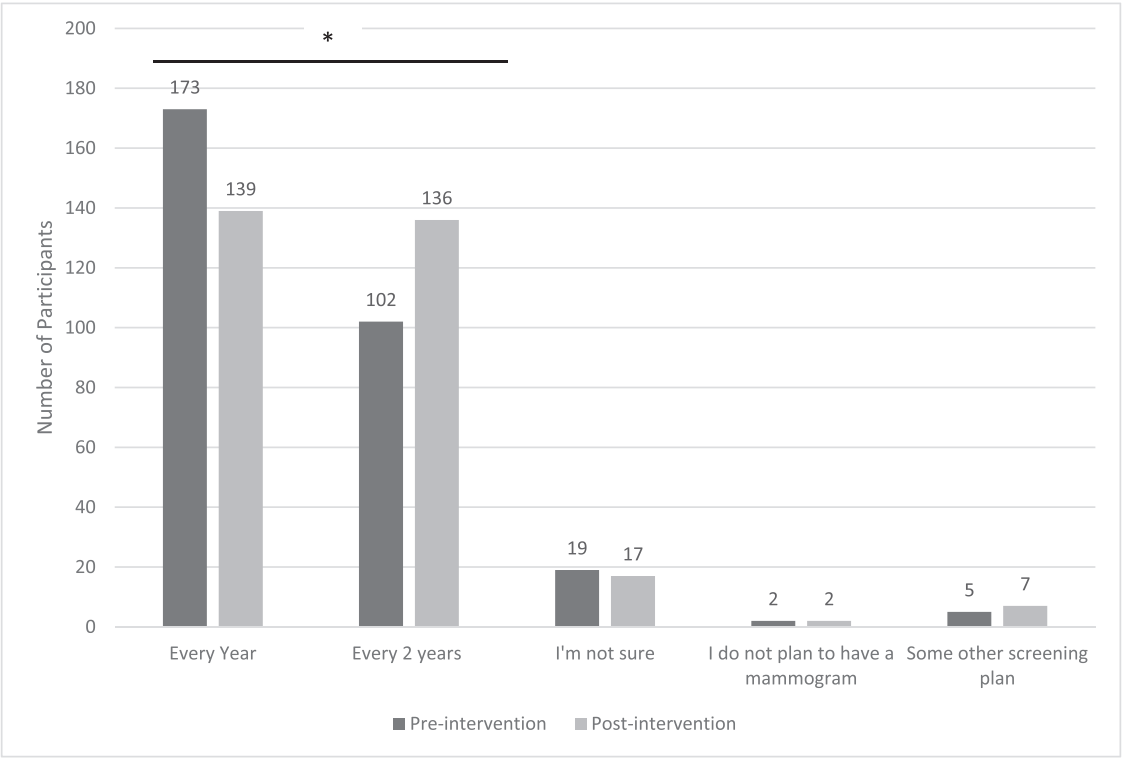


Fig. 2. Screening Frequency Preference Pre vs. Post Intervention. Note. The horizontal bar indicates that significant differences were observed between participants pre-intervention and post-intervention preferences to screen every year or screen every 2 years. After reading information about the new guideline change, participants were significantly more likely to want to screen biennially. *p < 0.001.

what information they personally felt was important. These data are described in Fig. 4.

4. Discussion

This study explored how the 2024 USPSTF mammography guideline

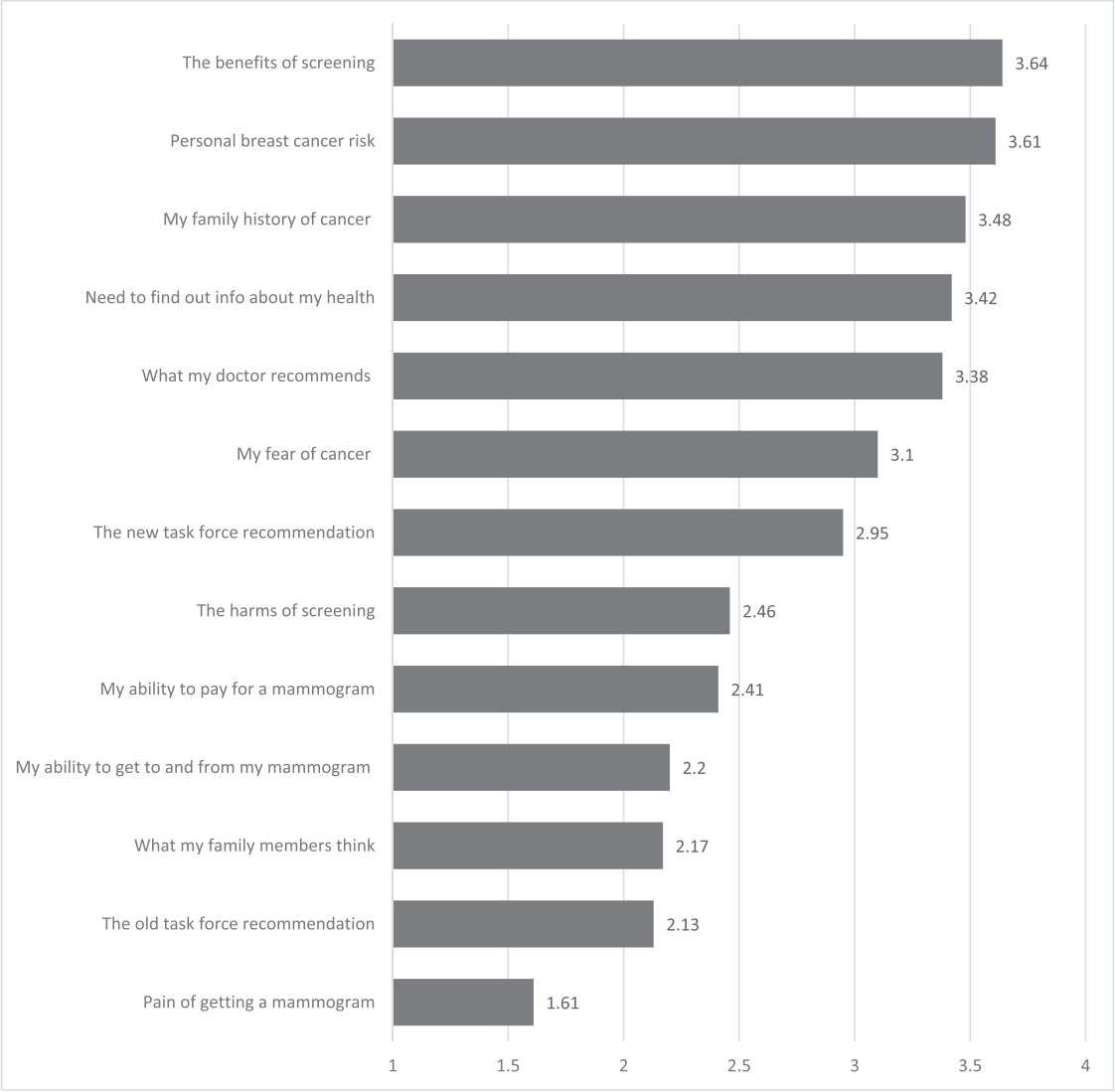


Fig. 3. Mean Importance Ratings of Different Factors Influencing Participant Screening Preferences. Note. Scale ranging from 1 =not at all important, 2 =slightly important, 3 =moderately important, 4 =very important.

influenced women's screening preferences, and their beliefs about what information women should be given prior to screening. In a group of women in their forties, who had been informed of the prior USPSTF guideline and the benefits and harms of screening, most initially wanted to screen at their current age and fewer than half wanted to screen biennially. After being exposed to patient-facing information about the new guideline, significantly more women wanted to screen at their current age and biennially (vs. annually). These data suggest that the clear communication of population-level recommendations can lead women to want to screen more consistently with the guideline. Thus, it is possible that more effective guideline dissemination could lead to more guideline-concordant screening among women of screening age.

Importantly, breast cancer screening preferences involve two distinct decision points, when to initiate screening and how often to screen. While the 2024 USPSTF BCS guideline primarily updated the recommended age of initiation, we observed a large shift in screening frequency preference (from annual to biennial screening). One plausible explanation is that participants baseline annual screening preference may not have reflected adherence to the prior USPSTF recommendation alone, but instead may have reflected exposure to, or alignment with, other screening guidance, such as the American Cancer Society that recommends annual screening for women 45–54, or individualized clinician recommendations. Within this context, exposure to the updated USPSTF recommendation, and the rationale underlying it, may have prompted women to reconsider both screening initiation age and frequency.

Our data also found that participants emphasized the importance of receiving detailed, evidence-based information when making screening decisions. Specifically, women rated information about the benefits of screening, their personal cancer risk, and their family history of cancer as being the particularly influential factors on their screening preferences. These findings are consistent with results from a nationally representative study of screening preferences. [14] The 2024 USPSTF guideline recommends that all women begin screening at age 40 and does not explicitly call for informed decision-making, unlike the previous guideline. [1] Nonetheless, women enrolled in this study also indicated that it was important for women to be informed about screening benefits, false positives, and overdiagnosis, as well as how often those outcomes happen. Thus, these data also indicate that women still view being fully informed about BCS harms as moderately to very important, even within the context of a simplified BCS guideline.

There has been some debate among clinicians and scientists about whether women should be informed about the harms of screening. Specifically, some have "strongly disagreed" that there is a close balance

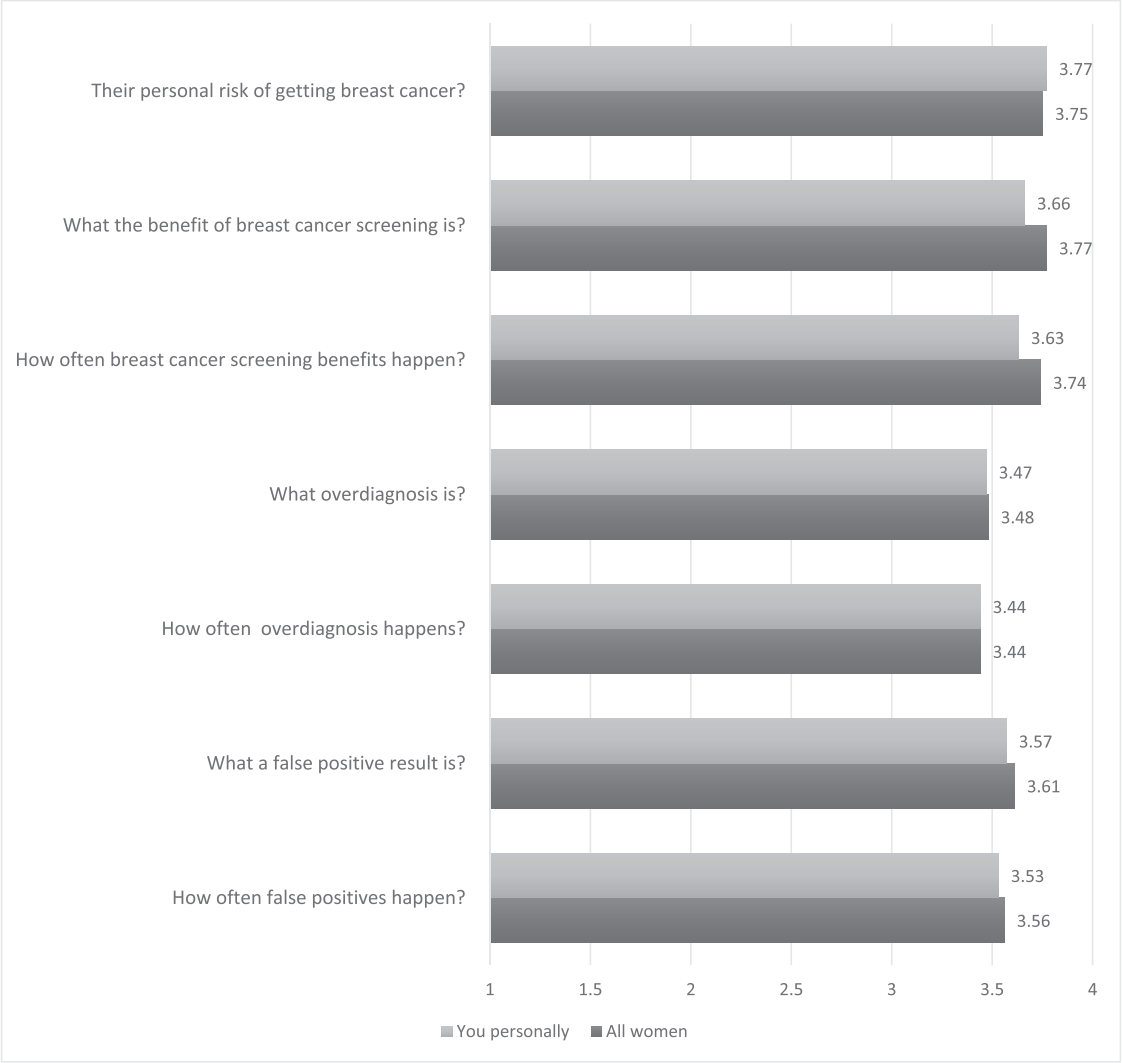


Fig. 4. Mean Ratings of What Women Think is Important For All Women to Know Before BCS. Note. Scale ranging from 1 =not at all important, 2 =slightly important, 3 =moderately important, 4 =very important.

between the harms and benefits of screening and posited that the dissemination of this information may potentially harm women by deterring them from screening. [17] Others have suggested that the dissemination of this information is in "effort to reduce access and participation in breast cancer screening" among women of screening eligible age. [18] Our data address these concerns by not only suggesting that women see value in being informed about both screening benefits and harms but also view false positives and overdiagnosis as important for all women to be educated about prior to getting screened. Further, these data demonstrate that many women who are fully informed about screening still prefer to screen in accordance with guidelines.

Overall, this study indicates that women want evidence-based information about both BCS benefits and harms. Public health communication strategies should reflect these preferences by ensuring that women are equipped with the information they want and need to make informed choices about screening, even when the guidelines do not explicitly call for such measures.

4.1. Limitations

Outcome measures were self-reported and based on reported intention rather than behavior. While intentions are often predictive of future action, we cannot determine from this study whether participants adhered to their reported screening preferences. Although a strength of this study is that the sample was drawn from a uniquely well-informed group of women enrolled in a primary care practice who had been previously enrolled in our parent study, it is possible that this group may be more engaged than the general population. Relatedly, participants were also explicitly informed that the USPSTF breast cancer screening guideline had recently changed and were provided explicit information about how it changed. In routine clinical care, many women learn screening recommendations during clinical encounters with their medical providers without their physicians explicitly referencing prior recommendations. As such, it is possible that awareness of the recent guideline change may influence women's screening preferences and informational needs and thus our findings may be most applicable to contexts in which a guideline update has been effectively disseminated to the public. Finally, the sample was drawn from two academic health systems and may not reflect the views of the larger U.S. population in terms of race, ethnicity, educational attainment, and access to preventative care.

4.2. Conclusions

Exposure to information about the new USPSTF BCS guideline led many women to adjust their preferences for BCS in a way that aligns

more closely with the new recommendations. Further, even in the absence of explicit language promoting informed decision-making, women continued to believe being informed about BCS benefits and harms remains important. These findings underscore the importance of transparency in public health messaging. Ensuring that women are provided with complete and comprehensive screening information may foster an environment for adherence to screening recommendations, while also leaving space for individual autonomy.

4.3. Practice implications

These data highlight the importance of providing screening eligible women clear and directive information about the 2024 USPSTF BCS guideline. Doing so, may lead women to shift their screening preference towards guideline-concordant decisions. At the same time, our findings suggest that, despite the removal of explicit informed decision-making language from the 2024 guideline, women still believe it is important to be educated about both the harms and benefits of BCS. Thus, to prioritize patient-centered care, providers may consider integrating brief, informed screening discussions about the harms, benefits, and patients' personal risk, into routine clinical encounters when addressing BCS.

CRediT authorship contribution statement

Kirsten McCaffery: Writing – review & editing, Methodology, Conceptualization. **Grant Yoder:** Writing – review & editing, Project administration, Investigation, Formal analysis. **Tamar Parmet:** Writing – original draft, Project administration, Methodology, Formal analysis, Conceptualization. **Laura D. Scherer:** Writing – review & editing, Supervision, Methodology, Investigation, Funding acquisition, Conceptualization. **Brad Morse:** Writing – review & editing, Methodology, Conceptualization. **Channing Tate:** Writing – review & editing, Methodology, Conceptualization. **Joseph N. Cappella:** Writing – review & editing, Methodology, Conceptualization. **Carmen L. Lewis:** Writing – review & editing, Methodology, Conceptualization. **Marilyn M. Schapira:** Writing – review & editing, Methodology, Conceptualization. **Jolyn Hersch:** Writing – review & editing, Methodology, Conceptualization.

Funding

This study was funded through NIH/NCI R37CA254926.

Clinical Trials.gov registration

NCT05838417.

Declaration of Competing Interest

The authors declare that they have no known competing financial interests or personal relationships that could have appeared to influence the work reported in this paper.

Appendix A. Supporting information

Supplementary data associated with this article can be found in the online version at doi:10.1016/j.pec.2026.109581.

Data availability

The data underlying this article are available in the article and in its online supplementary material.

References

[1] US Preventive Services Task Force. Screening for Breast Cancer: US Preventive Services Task Force Recommendation Statement. JAMA 2024;331(22):1918–30. https://doi.org/10.1001/jama.2024.5534.
[2] US Preventive Services Task Force. Screening for breast cancer: U.S. preventive services task force recommendation statement. w-236 Ann Intern Med Nov 17 2009;151(10):716–26. https://doi.org/10.7326/0003-4819-151-10-200911170-00008.
[3] Siu AL. Screening for Breast Cancer: U.S. Preventive Services Task Force Recommendation Statement. Ann Intern Med Feb 16 2016;164(4):279–96. https://doi.org/10.7326/m15-2886.
[4] United States Preventive Services Task Force. Screening for breast cancer: U.S. preventive services task force recommendation statement. w-236 Ann Intern Med Nov 17 2009;151(10):716–26. https://doi.org/10.7326/0003-4819-151-10-200911170-00008.
[5] U.S. Preventive Services Task Force. Shared Decision Making About Screening and Chemoprevention. Accessed July 9, 2024. ⟨https://www.uspreventiveservicestaskforce.org/uspstf/about-uspstf/methods-and-processes/shared-decision-making-about-screening-and-chemoprevention#:~:text=Whenever%20decisions%20about%20preventive%20services,partnership%20is%20central%20to%20decisionmaking⟩.
[6] Davidson KW, Mangione CM, Barry MJ, et al. Collaboration and shared decision-making between patients and clinicians in preventive health care decisions and us preventive services task force recommendations. Jama Mar 22 2022;327(12):1171–6. https://doi.org/10.1001/jama.2022.3267.
[7] National Cancer Institute. Cancer Stat Facts: Female Breast Cancer. National Cancer Institute, Surveillance, Epidemiology, and End Results Program; 2025. ⟨https://seer.cancer.gov/statfacts/html/breast.html⟩.
[8] DeSantis CE, Ma J, Gaudet MM, et al. Breast cancer statistics, 2019. CA Cancer J Clin Nov 2019;69(6):438–51. https://doi.org/10.3322/caac.21583.
[9] American Cancer Society. Cancer Facts and Figures. 2025. ⟨https://www.cancer.org/content/dam/cancer-org/research/cancer-facts-and-statistics/annual-cancer-facts-and-figures/2024/2024-cancer-facts-and-figures-acs.pdf?utm_source=chatgpt.com⟩.
[10] Kiviniemi MT, Hay JL. Awareness of the 2009 US preventive services task force recommended changes in mammography screening guidelines, accuracy of awareness, sources of knowledge about recommendations, and attitudes about updated screening guidelines in women ages 40–49 and 50+. 2012/10/24 BMC Public Health 2012;12(1):899. https://doi.org/10.1186/1471-2458-12-899.
[11] Allen JD, Bluethmann SM, Sheets M, et al. Women's responses to changes in U.S. Preventive Task Force's mammography screening guidelines: results of focus groups with ethnically diverse women. BMC Public Health Dec 12 2013;13:1169. https://doi.org/10.1186/1471-2458-13-1169.
[12] Qin X, Tangka FK, Guy Jr GP, Howard DH. Mammography rates after the 2009 revision to the United States preventive services task force breast cancer screening recommendation. Cancer Causes Control Jan 2017;28(1):41–8. https://doi.org/10.1007/s10552-016-0835-1.
[13] Scherer L.D., Lewis C.L., Cappella J.N., et al. Skepticism Towards Breast Cancer Screening Evidence Over Time. (Under Review);
[14] Scherer LD, Lewis CL, McCaffery K, et al. Mammography screening preferences among screening-eligible women in their 40s: A national U.S. survey. Ann Intern Med Aug 2024;177(8):1069–77. https://doi.org/10.7326/m23-3325.
[15] Scherer LD, Suresh K, Lewis CL, et al. Assessing and understanding reactance, self-exemption, disbelief, source derogation and information conflict in reaction to overdiagnosis in mammography screening: scale development and preliminary validation. Med Decis Mak Oct-Nov 2023;43(7-8):789–802. https://doi.org/10.1177/0272989x231195603.
[16] United States Preventive Services Task Force. Frequently Asked Questions. 2024. ⟨https://www.uspreventiveservicestaskforce.org/uspstf/recommendation-topics/breast-cancer-screening-faqs⟩.
[17] Bennett D, Chiu S. Michelle L. Mammography screening preferences among screening-eligible women in their 40s. Ann Intern Med 2025;178(7):1060. doi: 10.7326/annals-25-01108%m 40658965.
[18] Kopans DB. The continuing effort to reduce access and participation in breast cancer screening. Ann Intern Med 2025;178(7):1060.